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EZIVAS (EZETIMIBE TABLETS 10MG)

ACTIVE INGREDIENT
Each uncoated tablet contains 10mg of Ezetimibe.

DOSAGE FORM
Oral Tablets

PRODUCT DESCRIPTION
White to off white, capsule shaped, flat faced bevel edged tablets debossed with 'I' on one side and '83' on the other side.

PHARMACODYNAMICS

Pharmacotherapeutic group: Other lipid modifying agents. ATC code: C10A X09

Mechanism of action

EZIVAS is orally active and potent, with a unique mechanism of action that differs from other classes of cholesterol-reducing compounds (e.g., statins, bile acid sequestrants [resins], fibric acid derivatives, and plant stanols). The molecular target of ezetimibe is the sterol transporter, Niemann-Pick C1-Like 1 (NPC1L1), which is responsible for the intestinal uptake of cholesterol and phytosterols.

Ezetimibe localizes at the brush border of the small intestine and inhibits the absorption of cholesterol, leading to a decrease in the delivery of intestinal cholesterol to the liver. This causes a reduction of hepatic cholesterol stores and an increase in clearance of cholesterol from the blood. Ezetimibe does not increase bile acid excretion (like bile acid sequestrants) and does not inhibit cholesterol synthesis in the liver (like statins).

PHARMACOKINETIC PROPERTIES

Absorption

After oral administration, ezetimibe is rapidly absorbed and extensively conjugated to a pharmacologically active phenolic glucuronide (ezetimibe-glucuronide). Mean maximum plasma concentrations (C_{max}) occur within 1 to 2 hours for ezetimibe-glucuronide and 4 to 12 hours for ezetimibe. The absolute bioavailability of ezetimibe cannot be determined as the compound is virtually insoluble in aqueous media suitable for injection.
Concomitant food administration (high fat or non-fat meals) had no effect on the oral bioavailability of ezetimibe when administered as EZIVAS 10mg tablets. EZIVAS can be administered with or without food.

Distribution

Ezetimibe and ezetimibe-glucuronide are bound 99.7% and 88 to 92% to human plasma proteins, respectively.

Metabolism

Ezetimibe is metabolized primarily in the small intestine and liver via glucuronide conjugation (a phase II reaction) with subsequent biliary excretion. Minimal oxidative metabolism (a phase I reaction) has been observed in all species evaluated. Ezetimibe and ezetimibe-glucuronide are the major drug-derived compounds detected in plasma, constituting approximately 10 to 20 % and 80 to 90 % of the total drug in plasma, respectively. Both ezetimibe and ezetimibe-glucuronide are slowly eliminated from plasma with evidence of significant enterohepatic recycling. The half-life for ezetimibe and ezetimibe-glucuronide is approximately 22 hours.

Elimination

Following oral administration, total ezetimibe accounted for is approximately 93% in plasma. Approximately 78% and 11% are recovered in the feces and urine, respectively, over a 10-day period. After 48 hours of oral administration, there were no detectable levels of ezetimibe in the plasma.

Special populations

Pediatric patients:
The absorption and metabolism of ezetimibe are similar between children and adolescents (10 to 18 years) and adults. Based on total ezetimibe, there are no pharmacokinetic differences between adolescents and adults. Pharmacokinetic data in the pediatric population < 10 years of age are not available. Clinical experience in pediatric and adolescent patients (ages 9 to 17) has been limited to patients with HoFH or sitosterolemia.

Geriatric Patients:
Plasma concentrations for total ezetimibe are about 2-fold higher in the elderly (≥ 65 years) than in the young (18 to 45 years). LDL-C reduction and safety profile are comparable between elderly and young subjects treated with ezetimibe. Therefore, no dosage adjustment is necessary in the elderly.

Hepatic Insufficiency:
No dosage adjustment is necessary for patients with mild hepatic insufficiency. Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe (Child Pugh score > 9) hepatic insufficiency, EZIVAS is not recommended in these patients.

Renal Insufficiency:
No dosage adjustment is necessary for renally impaired patients.

Gender:
No dosage adjustment is necessary on the basis of gender.

Race:
Based on a meta-analysis of pharmacokinetic studies, there were no pharmacokinetic differences between Blacks and Caucasians.

INDICATION

Primary Hypercholesterolaemia

EZIVAS administered alone, or with an HMG-CoA reductase inhibitor (statin), is indicated as adjunctive therapy to diet in patients with primary (heterozygous familial and non-familial) hypercholesterolaemia.

EZIVAS, administered in combination with fenofibrate, is indicated as adjunctive therapy to diet for the reduction of elevated total-C, LDL-C, Apo B, and non-HDL-C in patients with mixed hyperlipidemia.

Prevention of Cardiovascular Events

EZIVAS, administered with a statin, is indicated to reduce the risk of cardiovascular events (cardiovascular death, nonfatal myocardial infarction, nonfatal stroke, hospitalization for unstable angina, or need for revascularization), in patients with coronary heart disease (CHD) and a history of acute coronary syndrome (ACS).

Homozygous Familial Hypercholesterolaemia (HoFH)

EZIVAS, administered with a statin, is indicated for patients with HoFH. Patients may also receive adjunctive treatments (e.g., LDL apheresis).

Homozygous Sitosterolaemia (Phytosterolaemia)

EZIVAS is indicated for the reduction of elevated sitosterol and campesterol levels in patients with homozygous familial sitosterolaemia.

Recommended dose

The patient should be on an appropriate lipid-lowering diet and should continue to be on this diet during treatment with EZIVAS.

Use in Patients with Primary Hypercholesterolemia

The recommended dose of EZIVAS is 10 mg once daily, used alone, with a statin or with fenofibrate. EZIVAS can be administered at any time of the day, with or without food.

EZIVAS may be administered with a statin (in patients with primary hypercholesterolemia) or with fenofibrate (in patients with mixed hyperlipidemia) for incremental effect. For convenience, the daily dose of EZIVAS may be taken at the same time as the statin or fenofibrate, according to the dosing recommendations for the respective medications.

If EZIVAS 10mg tablets are used in combination with a statin therapy, the dosage instructions for that particular statin should be consulted.

When initiating lipid lowering treatment, which includes EZIVAS 10mg Tablets and a statin in combination, the indicated usual initial dose of that particular statin should be used or the already established higher statin dose should be continued.

If the statin dose is to be increased for the first time or further, the dosage instructions of that particular statin should be followed (such as dose increase only after at least 4 weeks of regular use of the combination without any change).

The stepwise increase of the statin dose in combination treatment results in a relatively small additional decrease of LDL-C but increases the risk of dose-related adverse events of the statin. This has to be considered for the risk-benefit-assessment when the statin dose is considered.

Use in Patients with Coronary Heart Disease

Therapy with a Statin
For incremental cardiovascular event reduction in patients with coronary heart disease, EZIVAS 10 mg may be administered with a statin with proven cardiovascular benefit.

Use in the Elderly

No dosage adjustment is required for elderly patients. However, greater sensitivity of some older individuals cannot be ruled out.

Use in Pediatric Patients

Children and adolescents ≥ 10 years: No dosage adjustment is required.
Children < 10 years: Treatment with EZIVAS is not recommended.

Use in Hepatic Impairment

No dosage adjustment is required in patients with mild hepatic insufficiency (Child Pugh score 5 to 6). Treatment with EZIVAS is not recommended in patients with moderate (Child Pugh score 7 to 9) or severe (Child Pugh score > 9) liver dysfunction.

Use in Renal Impairment

No dosage adjustment is required for renally impaired patients.

Co-administration with bile acid sequestrants

Dosing of EZIVAS should occur either ≥ 2 hours before or ≥ 4 hours after administration of a bile acid sequestrant.

MODE OF ADMINISTRATION
Oral route of administration

CONTRAINDICATIONS
Hypersensitivity to any component of this medication.
The combination of EZIVAS with a statin is contraindicated in patients with active liver disease or unexplained persistent elevations in serum transaminases.

All statins are contraindicated in pregnant and nursing women. When EZIVAS is administered with a statin in a woman of childbearing potential, refer to the pregnancy category and product labeling for the statin.

WARNINGS AND PRECAUTIONS
When EZIVAS is to be administered with a statin or with fenofibrate, please refer to the package insert for that particular medication.

Liver Enzymes

When EZIVAS is co-administered with a statin, liver function tests should be performed at initiation of therapy and according to the recommendations of the statin.

Skeletal Muscle

There is no excess of myopathy or rhabdomyolysis associated with ezetimibe. However, myopathy and rhabdomyolysis are known adverse reactions to statins and other lipid-lowering drugs.

In post-marketing experience with ezetimibe, cases of myopathy and rhabdomyolysis have been reported regardless of causality. Most patients who developed rhabdomyolysis were taking a statin prior to initiating ezetimibe. However, rhabdomyolysis has been reported very rarely with ezetimibe monotherapy and very rarely with the addition of ezetimibe to agents known to be associated with increased risk of rhabdomyolysis. All patients starting therapy with ezetimibe should be advised of the risk of myopathy and told to report promptly any unexplained muscle pain, tenderness or weakness. Ezetimibe and any statin that the patient is taking comcomitantly should be immediately discontinued if myopathy is diagnosed or suspected. The presence of these symptoms and a creatine phosphokinase (CPK) level >10 times the ULN indicates myopathy.

Hepatic Insufficiency

Due to the unknown effects of the increased exposure to ezetimibe in patients with moderate or severe hepatic insufficiency, EZIVAS is not recommended in these patients.

Dimensions	170 x 300 mm (Pre Fold: 170 x 30 mm)
Customer/Country	Camber / Malaysia
Spec	Bible Paper 40 GSM
Pantone Colours	Black
Version No.	01
If Any	Note: Do not change pharma codes position

The above table format is for specification purpose only, Refer approval specification for final details.

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Fibrates
The co-administration of ezetimibe with fibrates other than fenofibrate has not been studied. Therefore, co-administration of EZIVAS and fibrates (other than fenofibrate) is not recommended.

Fenofibrate
If cholelithiasis is suspected in a patient receiving EZIVAS and fenofibrate, gallbladder studies are indicated, and alternative lipid-lowering therapy should be considered.

Cyclosporine
Caution should be exercised when initiating EZIVAS in the setting of cyclosporine. Cyclosporine concentrations should be monitored in patients receiving EZIVAS and cyclosporine.

Anticoagulants
If EZIVAS is added to warfarin, another coumarin anticoagulant, or fluindione, the International Normalized Ratio (INR) should be appropriately monitored.

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION
Ezetimibe does not induce cytochrome P450 drug metabolizing enzymes. No clinically significant pharmacokinetic interactions have been observed between ezetimibe and drugs known to be metabolized by cytochromes P450 1A2, 2D6, 2C8, 2C9, and 3A4, or N-acetyltransferase.
Ezetimibe had no effect on the pharmacokinetics of dapsone, dextromethorphan, digoxin, oral contraceptives (ethinyl estradiol and levonorgestrel), glipizide, tolbutamide, or midazolam during co-administration. Cimetidine, co-administered with ezetimibe, had no effect on the bioavailability of ezetimibe.

Antacids: Concomitant antacid administration decreased the rate of absorption of ezetimibe but had no effect on the bioavailability of ezetimibe. This decreased rate of absorption is not considered clinically significant.

Cholestyramine: Concomitant cholestyramine administration decreased the mean AUC of total ezetimibe (ezetimibe + ezetimibe glucuronide) approximately 55 %. The incremental LDL-C reduction due to adding EZIVAS to cholestyramine may be lessened by this interaction.

Cyclosporine: Caution should be exercised when initiating ezetimibe in the setting of cyclosporine. Cyclosporine concentrations should be monitored in patients receiving ezetimibe and cyclosporine.

Fibrates: Fibrates may increase cholesterol excretion into the bile, leading to cholelithiasis. Co-administration of EZIVAS with fibrates (other than fenofibrate) is not recommended until use in patients is studied.

Fenofibrate: Concomitant fenofibrate administration increased total ezetimibe concentrations approximately 1.5-fold. However, this increase is not considered clinically significant.

Gemfibrozil: Concomitant gemfibrozil administration increased total ezetimibe concentrations approximately 1.7-fold. However, this increase is not considered clinically significant.

Statins: No clinically significant pharmacokinetic interactions were seen when ezetimibe was co-administered with atorvastatin, simvastatin, pravastatin, lovastatin, fluvastatin or rosuvastatin.

Anticoagulants: Concomitant administration of ezetimibe (10 mg once daily) had no significant effect on bioavailability of warfarin and prothrombin time. There have been post-marketing reports of increased International Normalized Ratio in patients who had ezetimibe added to warfarin or fluindione. Most of these patients were also on other medications.

PREGNANCY AND LACTATION
Pregnancy
EZIVAS should be used during pregnancy only if the potential benefit justifies the risk to the fetus.

When EZIVAS is to be administered with a statin, please refer to the Package Insert for that particular statin.

Breast-feeding
It is not known whether ezetimibe is excreted into human breast milk, therefore, EZIVAS should not be used in nursing mothers unless the potential benefit justifies the potential risk to the infant.

SIDE EFFECTS
EZIVAS administered alone
Investigations
Uncommon: ALT and/or AST increased; blood CPK increased; gamma-glutamyltransferase increased; liver function test abnormal

Respiratory, Thoracic and Mediastinal Disorders
Uncommon: cough

Gastrointestinal Disorders
Common: abdominal pain; diarrhea; flatulence
Uncommon: dyspepsia; gastroesophageal reflux disease; nausea

Musculoskeletal and Connective Tissue Disorders
Uncommon: arthralgia; muscle spasms; neck pain

Metabolism and Nutrition Disorders
Uncommon: decreased appetite

Vascular Disorders
Uncommon: hot flush; hypertension

General Disorders and Administration Site Condition
Common: fatigue
Uncommon: chest pain; pain

EZIVAS co-administered with a statin
Investigations
Common: ALT and/or AST increased

Nervous System Disorders
Common: headache
Uncommon: paresthesia

Gastrointestinal Disorders
Uncommon: dry mouth; gastritis

Skin and Subcutaneous Tissue Disorders
Uncommon: pruritus; rash; urticaria
Musculoskeletal and Connective Tissue Disorders
Common: myalgia
Uncommon: back pain; muscular weakness; pain in extremity

General Disorders and Administration Site Condition
Uncommon: asthenia; edema peripheral

EZIVAS co-administered with fenofibrate
Gastrointestinal Disorders
Common: abdominal pain

Post-marketing Experience
The following adverse reactions have been reported in post-marketing experience, regardless of causality assessment:
Blood and lymphatic system disorders: thrombocytopaenia
Nervous system disorders: dizziness; paraesthesia
Gastrointestinal disorders: nausea; pancreatitis; constipation
Skin and subcutaneous tissue disorders: erythema multiforme
Musculoskeletal and connective tissue disorders: arthralgia; myalgia; myopathy/rhabdomyolysis

General disorders and administration site conditions: asthenia
Immune system disorders: Hypersensitivity reactions, including anaphylaxis, angioedema, rash, and urticaria
Hepatobiliary disorders: hepatitis; cholelithiasis; cholecystitis Psychiatric disorders: depression

SYMPTOMS AND TREATMENT OF OVERDOSE
A few cases of overdosage with ezetimibe have been reported; most have not been associated with adverse experiences. Reported adverse experiences have not been serious. In the event of an overdose, symptomatic and supportive measures should be employed.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES
There have been side effects reported with ezetimibe that may affect your ability to drive or operate machinery. Individual responses to EZIVAS may vary.

PHARMACEUTICAL PARTICULARS
List of excipients:
Lactose Monohydrate, Microcrystalline Cellulose, Silica Colloidal Anhydrous, Sodium Lauril Sulfate, Crossmellose Sodium, Hypromellose, Purified Water and Magnesium Stearate

Storage condition
Store below 30°C and protect from moisture. Store in the original package.

Packaging available:
A box of 3 x 10's Alu-Alu Blister Pack.

MANUFACTURED BY:
Hetero Labs Limited,
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